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ORIGINAL ARTICLE

# Parental Knowledge and Recognition of Preschool Manifestations of ADHD in Preterm Children: A Cross-Sectional Study

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## ABSTRACT

**Aim:** Preterm birth is a major risk factor for attention-deficit/hyperactivity disorder (ADHD). Early recognition is crucial for academic and social development. Preschool identification remains challenging as it relies heavily on parental observation. This study aimed to evaluate parental knowledge of ADHD and recognition of preschool ADHD symptoms in a high-risk preterm population.

**Methods:** Parents of children born preterm attending a tertiary outpatient clinic (January 2022–March 2023) completed a questionnaire in this cross-sectional study. The tool, validated by child psychiatry experts, assessed general ADHD knowledge and recognition of preschool manifestations. Responses were scored (*correct* = 1; *incorrect/unsure* = 0) and converted to a 0–100 scale.

**Results:** A total of 138 parents participated; 14% reported a family history of ADHD. Most participants had a university-level education (61%) and were employed (66%). The median total symptom recognition score was 38 (P25–P75: 19–52). Recognition was highest for hyperactivity [50 (0–50)] and lowest for impulsivity [25 (12–50)]. The median general ADHD knowledge score was 14 (14–28). Parents with a family history of ADHD showed significantly higher inattention recognition scores ($p = 0.035$), though total symptom recognition did not differ by family history ($p = 0.24$). General ADHD knowledge varied by occupation ($p = 0.01$), with teacher–parents scoring highest ($p = 0.004$). The overall model predicting general ADHD knowledge was significant ($p = 0.020$) in multivariable linear regression, but no single independent predictor remained significant.

**Conclusion:** Recognition of preschool ADHD symptoms, particularly impulsivity, is low among parents of children born preterm. Targeted educational interventions are required to enhance early identification and improve developmental outcomes.

## 1 | Introduction

Prematurity remains one of the most critical challenges in neonatal health. Despite significant advancements in neonatal intensive care and the improved survival of extremely preterm infants, the long-term neurodevelopmental sequelae and the associated socioeconomic burden represent a persistent global health priority [1]. Current estimates indicate a global preterm birth rate of approximately 10%, with preterm birth defined as delivery before 37 completed weeks of gestation, highlighting the scale of the population at risk [2]. Children born preterm exhibit significantly higher rates of cognitive and psychiatric morbidities, including a three to fourfold increased risk of developing attention-deficit/

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**Key Points**

- What is already known on this topic
  - Preterm birth is a well-established risk factor for neurodevelopmental disorders, specifically increasing the risk of attention-deficit/hyperactivity disorder (ADHD) by three to four times compared with term-born children.
  - Although early identification in the preschool years is critical for optimising long-term academic and social outcomes, it remains a clinical challenge.
  - Effective early detection in this high-risk population depends heavily on the accuracy of parental observations and their ability to report behavioural symptoms.
- What this paper adds
  - This study identifies significant knowledge gaps and low symptom recognition rates among parents of preterm children, with impulsivity being the least recognised manifestation.
  - While a professional background in education (e.g., being a teacher) correlates with higher general ADHD knowledge, a family history of ADHD does not significantly improve overall symptom recognition.
  - The findings highlight a critical need for targeted parental educational interventions and proactive screening protocols within neonatal follow-up clinics to bridge the gap in early identification.

hyperactivity disorder (ADHD) compared with their term-born peers [3, 4].

ADHD is a complex neurodevelopmental disorder defined by persistent patterns of inattention, hyperactivity and impulsivity [3]. While the global prevalence in the paediatric population is estimated between 3.4% and 5.3%, these rates are substantially higher in preterm cohorts [3]. Although ADHD symptoms may emerge as early as the preschool years, recognition in this period is challenging because opportunities for structured observation are limited and early detection often depends on caregivers' observations of day-to-day functioning. Delayed recognition and management of ADHD are associated with suboptimal academic performance, impaired social integration and increased risks of peer victimisation or impulsive behaviours in later childhood [5–7].

Given that early intervention is the cornerstone of optimising developmental trajectories, understanding the baseline awareness of parents who serve as the primary observers is essential. While the epidemiological link between prematurity and ADHD is well-documented, there is a paucity of data regarding the parents' general ADHD knowledge and their ability to recognise core preschool ADHD symptoms. The present study aimed to evaluate parental knowledge of ADHD and identify specific deficits in symptom recognition among parents of children born preterm to address this gap. These findings provide a basis for targeted educational strategies in neonatal follow-up care [8].

## 2 | Methods

### 2.1 | Study Design and Participants

This cross-sectional study was conducted between 1 January 2022 and 15 March 2023 at a tertiary neonatal outpatient clinic. Parents of children born preterm were invited to participate during routine follow-up visits. Child age at the time of the survey was categorised as $<1$, 1–6 and $>6$ years, while gestational age at birth was categorised as $<28$, 28–34 and 35–36 weeks. The study protocol was approved by the Kocaeli University Non-Interventional Clinical Research Ethics Committee (Approval No: 2022/346). As this was an exploratory cross-sectional study, no a priori sample size calculation was conducted. All consecutive eligible parents attending the clinic during the study period were invited to participate. The final sample size reflects the total number of completed questionnaires. This study was conducted and reported in accordance with the Strengthening the Reporting of Observational Studies in Epidemiology (STROBE) statement [9].

### 2.2 | Data Collection and Survey Instrument

The survey assessed general ADHD knowledge and the ability to recognise symptoms in preschool children in general, rather than evaluating parents' perceptions of their own child. The survey instrument comprised a demographic section and two specialised sections evaluating ADHD-related knowledge. The tool was developed with input from two child psychiatry specialists and underwent pilot testing with 10 parents to ensure clarity.

- Section I (Symptom Recognition): Assessed the recognition of preschool ADHD manifestations across inattention, hyperactivity and impulsivity domains. Responses were recorded as 'Yes', 'No', or 'Not sure'.
- Section II (General Knowledge): Evaluated broader ADHD concepts using 'True', 'False', or 'Not sure' responses.

Representative items from each section are provided to enhance conceptual clarity. In the symptom recognition section, parents were asked to evaluate whether specific behaviours are indicative of ADHD in preschool children across three domains. For inattention, an example item was: 'Does the child appear not to listen when spoken to or fail to follow instructions unless prompted repeatedly?' For hyperactivity, an example item was: 'Does the child frequently make excessive noise during play or have difficulty remaining seated during activities such as meals?' For impulsivity, an example item was: 'Does the child interrupt others, speak without permission, or have difficulty waiting for their turn?' In the general knowledge section, parents responded to statements assessing broader ADHD concepts, such as: 'Children born preterm are at higher risk of developing ADHD' and 'Having a family history of ADHD increases the likelihood of ADHD in a child' Items also included non-specific behaviours and commonly misattributed features



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to capture potential misconceptions, such as sensory sensitivities (e.g., discomfort with clothing textures), which are not core diagnostic criteria for ADHD.

### 2.3 | Scoring

Correct responses were assigned a score of 1, while incorrect or 'Not sure' answers were scored as 0. Raw scores from both the symptom recognition and general knowledge sections were converted to a 0–100 scale for standardised interpretation. Higher scores indicated greater knowledge or better symptom recognition. Domain-specific scores (inattention, hyperactivity, impulsivity) and a composite total symptom recognition score were calculated. Internal consistency was robust, with Cronbach's $\alpha$ values of 0.93 for symptom recognition and 0.82 for general knowledge. Incomplete questionnaires were excluded from the final analysis.

### 2.4 | Statistical Analysis

Data analysis was performed using IBM SPSS Statistics for Windows, Version 26.0 (IBM Corp., Armonk, NY, USA). Participants with incomplete questionnaires were excluded prior to analysis. As only complete datasets were analysed, no statistical imputation procedures were performed. The normality of continuous variables (inattention, hyperactivity, impulsivity, total symptom recognition and general knowledge scores) was evaluated using the Shapiro–Wilk test alongside visual inspection of histograms and $Q$–$Q$ plots. As the scores exhibited a non-normal distribution, non-parametric tests were employed. Continuous variables are presented as median (25P–75P), and categorical variables as number (percentage). The Mann–Whitney $U$ test was used to compare knowledge scores between two-group independent variables. For independent variables with three categories, the Kruskal–Wallis $H$ test was applied. When the Kruskal–Wallis test indicated a significant difference, post hoc pairwise comparisons were performed using the Mann–Whitney $U$ test with Bonferroni correction (adjusted significance threshold $p = 0.016$). A two-tailed $p < 0.05$ was considered statistically significant. In addition, multivariable linear regression analysis was performed to examine independent associations with the general ADHD knowledge score. Occupation was entered using dummy variables, and model diagnostics included collinearity assessment and residual evaluation.

## 3 | Results

A total of 16 questionnaires were excluded due to incomplete responses. A total of 138 parents participated in the study, corresponding to a response rate of 82%. The majority of respondents were mothers (72%). Among the 138 parents who completed the survey, 14% ($n = 20$) reported a family history of ADHD, whereas only a small proportion of children (3%) had a prior diagnosis of ADHD. Regarding educational attainment, 11% ($n = 16$) had completed primary school and 61% ($n = 85$) university-level education. A total of 93 parents (66%) were employed and 22 (16%) reported a household income at or below the minimum wage. Additional sociodemographic characteristics are presented in Table 1.

The median total ADHD symptom recognition score was 38 (P25–P75: 19–52). Symptom recognition was highest for hyperactivity [median (P25–P75): 50 (0–50)] and lowest for impulsivity [median (P25–P75): 25 (12–50)], while the median inattention recognition score was 41 (P25–P75: 12–50). The median general ADHD knowledge score was 14 (P25–P75: 14–28) (Table 2). Knowledge levels appeared limited, as reflected by low median scores and substantial variability across domains, particularly for impulsivity.

Parents with a family history of ADHD demonstrated significantly higher inattention recognition scores compared with those without [median 54 (P25–P75: 33–58) vs. 33 (P25–P75: 10.5–50); $p = 0.035$]. However, family history did not

**TABLE 1** | Demographic characteristics of the participants.

| | ***N* = 138 (%)** |
|---|---|
| Gestational age at birth (weeks) | |
| ≤ 28 | 24 (17.7) |
| 28–34 | 56 (40.5) |
| 35–36 | 58 (41.8) |
| Parental age (years) | |
| ≤ 30 | 44 (31.6) |
| > 30 | 94 (68.4) |
| Occupation | |
| Unemployed | 45 (32.9) |
| Teacher | 35 (25.3) |
| Employed (other professions) | 58 (41.8) |
| Educational level | |
| Primary school | 16 (11) |
| Secondary school | 37 (26) |
| University | 85 (61) |
| Income level | |
| Below minimum wage | 22 (16) |
| < 2 × minimum wage | 72 (51.9) |
| < 3 × minimum wage | 47 (34.2) |
| Number of children | |
| 1 | 54 (39.2) |
| 2–3 | 79 (57) |
| > 3 | 5 (3.8) |
| Chronological age of the preterm-born child | |
| < 1 year | 47 (34.2) |
| 1–6 years | 51 (36.7) |
| > 6 years | 40 (29.1) |



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**TABLE 2** | Attention-deficit/hyperactivity disorder (ADHD) knowledge scores of the participants.

| **Total ADHD symptom recognition score, median (P25–P75)** | **38 (19–52)** |
|---|---|
| Inattention symptom recognition score, median (P25–P75) | 41 (12–50) |
| Hyperactivity symptom recognition score, median (P25–P75) | 50 (0–50) |
| Impulsivity symptom recognition score, median (P25–P75) | 25 (12–50) |
| General ADHD knowledge score, median (P25–P75) | 14 (14–28) |

**TABLE 3** | Multivariable linear regression analysis for general ADHD knowledge score.

| **Predictor** | **B (SE)** | **95% CI** | ***p*** |
|---|---|---|---|
| Occupation (employed in other professions vs. unemployed) | −3.98 (5.07) | −14.09 to 6.13 | 0.435 |
| Occupation (teacher vs. unemployed) | 10.60 (6.21) | −1.78 to 22.98 | 0.092 |
| Family history of ADHD | 10.26 (5.91) | −1.52 to 22.04 | 0.087 |
| Educational level | 2.33 (5.40) | −8.44 to 13.09 | 0.668 |
| Income level | 1.76 (6.14) | −10.48 to 13.99 | 0.775 |
| Parental age | −7.79 (4.57) | −16.90 to 1.33 | 0.093 |

*Note:* Model fit: $R^2 = 0.185$; adjusted $R^2 = 0.117$; $F(6,72) = 2.718$; $p = 0.020$. Educational level and income level were entered as ordinal variables (coded 1–3). Occupation was entered as dummy variables with unemployed as the reference.

significantly influence total symptom recognition ($p = 0.24$) or general ADHD knowledge ($p = 0.065$).

General ADHD knowledge scores varied significantly across occupational groups [H(2) = 9.301, $p = 0.010$]. Post hoc analysis showed that teacher–parents achieved higher general knowledge scores [median 28 (P25–P75: 14–42)] than parents in other professions [median 14 (P25–P75: 0–28); $p = 0.004$ after Bonferroni correction]. No significant differences were found between unemployed parents and other groups. Total symptom recognition scores were not affected by occupation ($p = 0.15$). No significant differences were observed in parental ADHD knowledge or symptom recognition scores across child age groups (all $p > 0.05$).

In multivariable linear regression analysis examining factors associated with general ADHD knowledge, the overall model was statistically significant [$R^2 = 0.185$; $F(6, 72) = 2.718$; $p = 0.020$] However, none of the individual predictors remained independently significant (Table 3).

## 4 | Discussion

This study demonstrates that parental awareness of ADHD, particularly general knowledge and recognition of preschool symptoms, is notably limited in families of children born preterm, a population at high neurodevelopmental risk. While hyperactivity was more readily identified, recognition scores for inattention and, most strikingly, impulsivity were considerably lower. This disparity suggests that while overt motor restlessness is easily perceived, the more nuanced executive dysfunctions of ADHD are frequently overlooked. The relatively low median scores suggest that parental knowledge and symptom recognition may be limited, particularly for less overt ADHD features such as impulsivity. In the absence of established cut-off values, the observed median scores falling below half of the theoretical maximum suggest that core ADHD-related concepts may not be adequately understood. The lack of association between child age and parental awareness suggests that knowledge gaps persist regardless of the child's developmental stage. These findings are consistent with previous studies demonstrating suboptimal parental knowledge and frequent misconceptions about ADHD in both community and clinical contexts [10–13]. Given the well-established increased risk of ADHD and related functional impairments among children born preterm, these knowledge gaps are particularly concerning [8, 14–15]. The observed 'floor effect' in general knowledge scores further indicates a profound deficit in conceptual understanding, which may hinder early help-seeking behaviours despite the increased risk in this cohort. Although the children of the participating parents varied in age, the questionnaire specifically targeted recognition of ADHD symptoms in preschool-aged children.

Parents with a family history of ADHD demonstrated significantly higher inattention symptom recognition. This partially contrasts with prior reports indicating that even parents of children diagnosed with ADHD may have limited overall knowledge [12, 13]. Differences in study populations may account for this discrepancy. Many previous studies were conducted in clinical samples in which families were navigating diagnostic uncertainty, stigma and treatment-related challenges, which may influence knowledge in complex ways. In contrast, parents in our cohort may have gained familiarity with inattentive behaviours through observation of affected relatives. Additionally, parenting a child born preterm may heighten vigilance towards subtle developmental concerns. However, family history was not associated with higher total symptom recognition scores, suggesting that increased awareness may be domain-specific rather than generalised across symptom domains. Although only a small proportion of children had a prior ADHD diagnosis (3%), family history was more common and was associated with domain-specific recognition. This suggests that familiarity with ADHD through relatives may improve recognition of certain symptoms but does not necessarily result in a comprehensive understanding of the disorder.

Parental ADHD awareness has been most frequently evaluated in developing settings, where studies commonly use investigator-developed questionnaires and consistently report low knowledge levels [10–13]. In high-income countries, research attention has increasingly shifted towards treatment adherence, stigma and healthcare-seeking behaviours, although



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parental knowledge and attitudes remain clinically relevant determinants of care pathways [15, 16]. Importantly, low parental knowledge has been associated with delayed help-seeking and reduced engagement with healthcare services [10, 12–13]. This may contribute to diagnostic delays, as the median age at ADHD diagnosis remains approximately 12 years [1, 17]. Improving parental awareness may therefore represent a modifiable target to support earlier identification and timely referral, particularly in high-risk populations.

Occupation was associated with general ADHD knowledge, with teacher–parents demonstrating higher knowledge scores than parents in other professions. This finding aligns with evidence that teachers often identify ADHD-related behaviours more frequently than parents, likely due to prolonged observation in structured environments and greater familiarity with age-expected behaviours across peer groups [18, 19]. The finding that teacher–parents possessed superior general knowledge but not necessarily better symptom recognition in their own children is noteworthy. It suggests that professional exposure provides a theoretical framework that does not automatically translate into clinical vigilance at home during the preschool years. This highlights that even 'educated' parents require specific guidance regarding the preterm-ADHD phenotype. The predominance of mothers in the sample may reflect caregiving roles and could influence reported awareness levels. In multivariable analysis, the overall model predicting general ADHD knowledge was statistically significant. However, individual predictors did not remain independently significant, suggesting that knowledge levels may be shaped by multiple overlapping sociodemographic and experiential factors rather than a single dominant determinant. The relatively high proportion of teacher–parents in the sample may reflect sampling bias and could have led to an overestimation of general ADHD knowledge in this cohort.

Our results position neonatal follow-up clinics as critical 'windows of opportunity'. Preterm children often exhibit a predominantly inattentive phenotype that remains 'silent' until school entry. Integrating structured educational tools such as brief brochures or targeted counselling into routine visits could mitigate the current diagnostic delay which often persists until late childhood. Early developmental interventions targeting attention control have shown feasibility in very preterm infants, supporting the potential value of early guidance and structured monitoring within follow-up services [20, 21].

The questionnaire used in this study included items addressing core preschool ADHD symptoms as well as behaviours that may overlap with co-occurring features or common misconceptions. Although sensory sensitivities and social communication differences are not core diagnostic criteria for ADHD, their inclusion was intended to capture broader misunderstanding and symptom overlap. Nevertheless, future work may benefit from distinguishing core symptom recognition items from misconception-focused items to improve interpretability.

Strengths of this study include its focus on a clinically relevant high-risk cohort within a tertiary preterm follow-up setting and its emphasis on preschool manifestations of ADHD, a developmental period in which recognition is particularly challenging. In addition, the questionnaire demonstrated high internal consistency ($\alpha = 0.93$ for symptom recognition and $\alpha = 0.82$ for general knowledge), supporting the reliability of the instrument. Limitations include the investigator-developed nature of the questionnaire and the need for further psychometric evaluation (e.g., construct validity and test–retest reliability), and the cross-sectional design precluding causal inference. Despite expert validation and pilot testing of the instrument, the cross-sectional and self-reported nature of the data along with single-center recruitment may have introduced reporting and selection bias. Despite these limitations, the sample size was comparable to that of similar parental awareness studies and provides novel insight into a population at increased neurodevelopmental risk [1]. Although conducted in a single tertiary care center, the consecutive recruitment strategy and inclusion of a heterogeneous preterm cohort enhance the clinical representativeness of the sample, supporting cautious extrapolation to similar neonatal follow-up settings.

In conclusion, parents of children born preterm demonstrated limited ADHD-related knowledge, particularly regarding inattention and impulsivity, alongside a marked floor effect in general conceptual understanding. Integrating brief, structured parent-focused education into routine preterm follow-up visits such as providing parents with simple information brochures or conducting brief counselling sessions about ADHD risk may improve early recognition and support timely referral for ADHD evaluation in this high-risk population. Future studies should evaluate whether parent-focused educational interventions delivered during follow-up improve knowledge and translate into earlier and more appropriate developmental assessment pathways.

### Author Contributions

**Ozge Serce Pehlevan:** conceptualisation, methodology, data analysis, writing – original draft preparation, writing – review and editing. **Burcu Kardas:** supervision, resources, investigation, writing – review and editing. **Kubra Yildiz:** conceptualisation, methodology, data collection, data entry, writing – review and editing.


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The authors have nothing to report.


### Ethics Statement

The study protocol was approved by the Kocaeli University Non-Interventional Clinical Research Ethics Committee (Approval No: 2022/346).

### Conflicts of Interest

The authors declare no conflicts of interest.

### Data Availability Statement

The data that support the findings of this study are available from the corresponding author upon reasonable request.

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## Supporting Information

Additional supporting information can be found online in the Supporting Information section. **Data S1:** STROBE Statement—Checklist of items that should be included in reports of observational studies.



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